

SPARTAN COACHING

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Why Your Census Number Is Not Arbitrary

What nobody tells hospice sales reps about
the business — and the people — they sell for.

*"A rep who understands why the number exists sells with conviction.
A rep who understands who depends on it never needs to be
motivated again."*

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BEFORE THE NUMBERS

The Covenant of Care

What a patient actually gives a hospice branch — and what the branch owes them in return.

At some point early in your hospice sales career, someone gave you a number. Eight admissions a month. Twelve. An ADC of 60 by Q3. What that number almost certainly did not come with was an explanation of where it came from, what it mathematically requires, or — most importantly — what it actually means for the people your branch exists to serve.

This document exists to provide that explanation. All of it. Not just the math — though the math is here, in full — but the human reality beneath it. Because the rep who understands why the number exists sells with a different kind of conviction. And the rep who understands who depends on that number never needs to be motivated again.

An Act of Trust Unlike Almost Any Other in Medicine

When a patient's physician says 'it's time to consider hospice,' something profound happens. A family that has been fighting — navigating treatment options, managing side effects, coordinating between specialists, trying to hold everything together — is asked to stop fighting and start receiving. They are asked to trust that a team of people they have never met will show up. Will be there. Will know what to do when the pain becomes unmanageable at 2 a.m. Will help them understand what is happening. Will sit with their father when no one else can bear to.

That is not an administrative enrollment. That is not the activation of a Medicare benefit. That is a covenant. A family handing the most sacred chapter of their lives to a team of strangers and trusting — without the ability to verify in advance — that those strangers will be worthy of it.

"A patient does not just enroll in a benefit. They entrust a team of people with the most intimate chapter of their life — and their death. The census is what makes it possible to honor that trust."

Your census — the count of patients actively on service at any given moment — is the financial expression of that covenant. It is what funds the team those patients trusted. It pays their salaries every two weeks. It covers the medications in their nursing bags, the equipment in the patient's room, the fuel in their cars, the staff training that keeps their clinical skills sharp. Without sufficient census, the team cannot exist at full capacity. Without that team at full capacity, the patients who trusted your branch do not receive the care they were promised.

The Most Influential People in the Dying Process

Hospice is not a building patients go to. It is a team that comes to them — in their home, their assisted living room, their nursing facility bed — and wraps around them with expert, compassionate, coordinated care. The RN Case Manager who visits twice a week and knows which medications are working and which aren't. The hospice aide who helps a man bathe with dignity on what may be one of his last mornings. The social worker who sits with a daughter who has been managing medications, navigating insurance, and holding the family together for months — and finally tells her that she does not have to do this alone anymore. The chaplain who shows up without an agenda and asks only what a person needs to say.

These are not support roles. They are not peripheral to the mission. They are the mission. They are the most influential people in the dying process — the ones who shape whether a patient's final weeks are marked by suffering or by peace, by fear or by comfort, by loneliness or by the felt presence of people who genuinely know how to be there. They determine whether a family member looks back on that time with gratitude or with regret. They are the reason hospice exists.

And they are only there — in full, at full capacity, fully funded — when the census is high enough to sustain them.

THE NUMBER IS NOT A QUOTA

The census goal you were given is not a business metric dressed up as a target. It is the minimum condition under which the most influential people in the dying process can actually do their jobs. Below that number, the team is incomplete. Above it, the branch can invest in keeping them, training them, and extending their reach into a community that needs them.

PART ONE

The People Your Census Sustains

Every line item in a hospice payroll is a person — with training, credentials, relationships, and the weight of walking into someone's home on the hardest day of their family's life.

Understanding your branch's cost structure begins with understanding these people — not as budget line items, but as the specific human beings whose presence makes hospice care what it is. Each role below represents a trained clinician or coordinator who has dedicated their professional life to being present with the dying. Their existence on your payroll is made possible by one thing: a census high enough to fund them.

The Clinical Team — Required by Medicare, Funded by Census

These roles are not optional. Medicare Conditions of Participation require every certified hospice to maintain this interdisciplinary team regardless of census. They are hired, credentialed, trained, and retained — and their salaries are paid every two weeks whether the branch has 15 patients or 80. As census grows, the scaling roles (RN Case Managers, Hospice Aides) grow with it — by federal requirement, not by choice. The branch cannot defer those hires without risking compliance and, more importantly, without failing the patients already enrolled.

CLINICAL & OPERATIONAL ROLE	WHAT THEY DO FOR EACH PATIENT	ANNUAL COST
Executive Director	Leads clinical operations, regulatory compliance, and team quality. Accountable for the standard of care every patient receives — and for the decisions that determine whether that standard can be maintained. Sets the culture that the clinical team carries into every patient's home.	\$140,000
Director of Clinical Services	Manages the clinical staff and ensures care quality is consistent across every patient on service. Bridges the gap between the office and the bedside. Catches problems before they reach patients. Reviews and approves every plan of care. Often the person a struggling clinician calls first.	\$110,000
RN Case Manager (×2 min; +1 per 12 patients)	The primary clinical contact for each patient. Visits twice weekly or more. Manages pain. Assesses symptoms. Adjusts medications. Coordinates the full care team. Communicates with the physician. Over weeks or months, becomes one of the most trusted people in a dying patient's life. Required by Medicare: one for every 12 patients on service. That ratio is not a preference. It is a federal condition of participation.	\$100,000 each

CLINICAL & OPERATIONAL ROLE	WHAT THEY DO FOR EACH PATIENT	ANNUAL COST
Hospice Aide (×2 min; +1 per 8 patients)	Provides personal care — bathing, hygiene, grooming, comfort — and preserves human dignity in the most intimate moments of a person's final days. Often the most frequent visitor. The team member who knows whether the patient slept, ate, or seems more anxious than yesterday. Required at one per 8 patients. Cannot be deferred without failing the people already enrolled.	\$50,000 each
Licensed Social Worker	Carries the weight that the family has been carrying alone. Navigates financial resources, insurance, facility relationships, and family dynamics. Helps a daughter stop being her father's medical coordinator and start being his daughter again. Facilitates advance care planning. Provides grief support before and after death. Often the team member who holds the family together through the hardest weeks of their lives.	\$75,000
Chaplain	Provides care for the dimension of dying that medicine cannot reach — the existential, the spiritual, the unfinished. Sits with whatever a patient needs to say: fear, regret, faith, doubt, old wounds, or silence. Does not require a religious framework. Shows up without an agenda. For many patients, the chaplain is the person they remember most clearly in their final days. The only clinician whose primary tool is presence.	\$70,000
After-Hours RN	The voice on the other end of the 2 a.m. phone call. When a patient's breathing changes and their family panics at midnight, this person answers. Assesses over the phone. Guides the family through what they're seeing. Determines whether an in-person visit is needed. Goes if it is. No patient faces a nighttime crisis without a trained clinician available — because this position makes that possible. When budget pressure eliminates this role, families call 911. Patients die in emergency rooms. That is the cost.	\$95,000
Weekend RN	Ensures care does not pause on Saturday and Sunday. A patient's need for pain management, symptom control, or crisis support does not observe business hours. This role ensures the clinical team is never truly off — because neither is the work. A branch that cannot fund weekend coverage has patients whose suffering goes unaddressed for 48 hours every week.	\$95,000

CLINICAL & OPERATIONAL ROLE	WHAT THEY DO FOR EACH PATIENT	ANNUAL COST
Medical Director (Contract)	Certifies hospice eligibility. Oversees symptom management protocols. Provides physician-level oversight of every patient's plan of care. Required by Medicare for every certified hospice. The clinical authority who ensures each patient receives medically appropriate, evidence-based comfort care — and who signs the documentation that ensures the branch remains in compliance.	\$75,000
Intake Coordinator	Receives referrals. Processes admissions. Coordinates the first clinical visit — often within hours of the call. Frequently the first human voice a patient's family hears after a physician says 'hospice.' That first interaction — its speed, its warmth, its competence — shapes the family's trust in the branch before a single clinician walks through the door. A slow or cold intake process loses patients who would have benefited. A strong one begins the healing immediately.	\$60,000
Secretary / Administrator	Scheduling, documentation, billing support, and office operations. The operational foundation that keeps the branch running — without which clinicians spend their time on paperwork instead of patients, schedules collapse, billing errors accumulate, and the care system slows from the inside.	\$55,000

Annual salary estimates reflect typical ranges for experienced, qualified clinicians. Scaling roles (RN Case Manager, Hospice Aide) grow with census by Medicare requirement. Medicare Conditions of Participation: 42 CFR §418.56.

The Scaling Imperative — Why Census Growth Is Not Optional

The Medicare requirement that governs staffing ratios is not a guideline. A new RN Case Manager is required for every 12 patients added to the census. A new Hospice Aide for every 8. These thresholds exist because research and clinical experience have established that beyond these ratios, care quality degrades — visits get shorter, assessments become less thorough, and clinicians begin carrying a caseload that compromises their ability to truly know each patient.

When census falls and the branch cannot sustain the team fully, the consequences are not administrative. They are human. Leadership delays replacing a departing RN. After-hours coverage gets stretched thin. An aide carries more patients than she should. The chaplain's hours get cut. Training budgets disappear. The clinical team feels the pressure even when no one names its source — and the patients enrolled feel it too, even when no one says it out loud.

"That is the human cost of insufficient census. It does not show up on a balance sheet. It shows up at the bedside."

The Sales Rep — The Economic Engine Behind the Mission

Every person on the list above — the nurse, the aide, the chaplain, the social worker, the coordinator answering the phone at midnight — has a job because someone built a census. That someone is you.

The hospice sales rep is not a peripheral function. Not a liaison. Not a relationship builder who passes names to intake. The rep is the economic engine that funds the mission. Every admission generated is a patient who receives expert, compassionate, coordinated end-of-life care instead of a hospital room or a frightened family navigating a crisis alone. Every referral relationship built and maintained is a pipeline of future patients who will receive that care.

You are not selling a Medicare benefit. You are connecting the people who need this team to the team that needs them. And every time that connection isn't made — every time a referral doesn't come through, a follow-up doesn't happen, a patient doesn't get enrolled — the team loses a member of the census that funds their ability to do their work. And a family loses something they didn't know they needed until it wasn't there.

PART TWO

The Daily Cost Reality

What it costs to care for each patient, every day — before a single salary is paid.

Revenue from Medicare's Routine Home Care rate arrives as a flat per-diem — the same daily amount whether the patient had a crisis visit last night or a routine aide visit this morning. Before that per-diem reaches payroll, a significant portion of it is consumed by the direct clinical costs incurred for each patient, each day. These costs are not administrative. They are the medications, equipment, supplies, and travel that happen at the bedside — the material expression of care.

The Revenue Structure — Understanding the Per-Diem

Medicare pays two Routine Home Care rates based on how long a patient has been enrolled in the current benefit period. The 2025 national base rates are:

2025 MEDICARE ROUTINE HOME CARE — NATIONAL BASE RATES

- Day 1–60: \$224.62 per patient per day — the higher rate for early-enrollment days
- Day 61+: \$177.27 per patient per day — approximately 21% lower for extended stays

These rates are adjusted by each region's wage index, so actual rates vary by market. For planning purposes, most branches use the national base rate as a conservative benchmark. Branches with higher average length of stay have a structurally lower blended revenue per day — which directly raises the census required to break even.

Variable Clinical Costs — What the Per-Diem Pays For First

Before payroll, before overhead, before any operating income, the branch must fund the direct clinical costs of caring for each patient each day. Every patient on census generates these costs every single day — they scale with census whether the branch is growing or contracting:

COST CATEGORY	WHAT IT COVERS	TYPICAL RANGE / DAY	BASE PRESET
Pharmacy	All medications prescribed as part of the hospice plan of care — pain management, anti-nausea, anxiety relief, comfort agents, wound care supplies. The single largest variable cost for most patients. Varies significantly with acuity: an oncology patient with complex pain management can reach \$80–\$100/day on pharmacy alone.	\$15 – \$60	\$22.00

COST CATEGORY	WHAT IT COVERS	TYPICAL RANGE / DAY	BASE PRESET
Durable Medical Equipment (DME)	Hospital beds, wheelchairs, commodes, oxygen concentrators, and bedside equipment that keeps patients safe and comfortable at home — without hospital-level infrastructure. Ensures patients can remain in their chosen setting.	\$6 – \$15	\$9.00
Medical Supplies	Gloves, dressings, incontinence products, wound care materials, catheters, and the clinical supply bag that goes into every patient's home on every visit. These supplies are often invisible but never optional.	\$6 – \$20	\$8.00
Travel & Transportation	Mileage reimbursement and drive-time costs for every RN, aide, social worker, and chaplain visit. In rural or geographically spread territories, this line item alone can significantly affect branch profitability.	\$4 – \$12	\$7.00
Other Direct Costs	Contracted therapy services (PT, OT, Speech), lab draws, interpreter services, and any per-patient expense not captured above.	\$2 – \$10	\$4.00
TOTAL VARIABLE COST PER PATIENT PER DAY	The baseline spend incurred the moment a patient is on service — before a single salary is paid.	\$33 – \$117	\$50.00

Base preset reflects national average acuity. Oncology, wound-heavy, or complex pain management patients can reach \$80–\$120/day in variable costs alone.

Contribution Margin — What Remains for the People in Part One

After subtracting all direct variable costs from the blended RHC per-diem, what remains is the contribution margin per patient per day. This is the money available to pay every person on the list in Part One. Every nurse, aide, social worker, chaplain, after-hours clinician, administrator, and leader. This is the number that determines whether the team can exist at full capacity — or whether something gets cut.

TYPICAL CONTRIBUTION MARGIN RANGE

\$120 – \$155 per patient per day

At national base RHC rates with standard variable costs and a typical length of stay, most hospice branches operate with a contribution margin between \$120 and \$155 per patient per day. The Spartan simulator calculates your exact figure based on your actual rates and cost assumptions.

This single number drives every financial threshold in your branch's profitability model. Every patient admitted generates this amount toward the fixed costs that sustain the clinical team — every day they are on service. Every patient not admitted is that amount per day, every day, that does not exist.

It is worth pausing on that last point. When a patient who would have benefited from hospice dies in a hospital instead — because no one made the referral, because the rep did not follow up, because the conversation never happened — the branch does not just lose revenue. A family loses the RN who would have called at 2 a.m. A patient loses the aide who would have helped them bathe with dignity. A daughter loses the social worker who would have told her she didn't have to do this alone.

"Building and maintaining census is not a sales function disconnected from clinical care. It is the precondition for clinical care to exist."

PART THREE

The Numbers That Should Have Been Explained on Day One

The math behind break-even, target margin, and why your census goal is exactly what it is.

Everything in the first two sections leads to this: the financial math that connects a specific census target to the ability to maintain the clinical team in full. These formulas are not complicated. They are simply never explained to the people — hospice sales reps — whose daily work determines whether the numbers come true.

The Chain From Census to Financial Sustainability

#	METRIC	WHAT IT MEANS AND HOW IT CONNECTS
1	Average Daily Census (ADC)	The number of patients actively on service on any given day. Every calculation in this document begins here. Each additional patient on census adds their contribution margin to the pool that funds the team — every single day.
2	Blended Revenue Per Day	ADC × the blended RHC rate, adjusted for length-of-stay mix. For average LOS > 60 days, the formula is: $((\text{Day1Rate} \times 60) + (\text{Day61Rate} \times (\text{LOS} - 60))) \div \text{LOS}$. Longer stays mean a lower blended rate — which raises the census required to break even.
3	Annual Revenue	ADC × Blended Revenue Per Day × 365. The total Medicare revenue the branch generates at a given census level for a full year. This is the top of the financial structure — every other line comes out of it.
4	Annual Variable Cost	ADC × Total Variable Cost Per Patient Per Day × 365. Pharmacy, DME, supplies, travel, and other direct clinical costs that scale with census. These are funded before payroll.
5	Annual Payroll	The sum of all staff salaries required at that census level, including the Medicare-mandated scaling roles (RN Case Managers, Hospice Aides) that must grow as census grows. This is typically the single largest fixed cost line.
6	Annual Overhead	Monthly non-payroll fixed costs (office, EMR, malpractice insurance, compliance, licensing, billing software) × 12. These costs do not change with census — the same whether the branch has 10 patients or 150.

#	METRIC	WHAT IT MEANS AND HOW IT CONNECTS
7	Annual Profit / (Loss)	Revenue – Variable Cost – Payroll – Overhead. The operating income remaining after all costs are covered. A negative number means the branch is losing money every patient-day. The clinical team is present and serving — but the business is drawing down capital that will eventually run out.
8	Operating Margin %	Annual Profit ÷ Annual Revenue × 100. The industry benchmark for a financially healthy hospice branch is 12–18%. Below 10% is considered fragile — the branch cannot absorb disruptions or invest in quality. Above 20% indicates a branch with capacity to expand, reinvest, and weather census swings.
9	Break-Even ADC	Annual Fixed Cost ÷ (Contribution Margin Per Day × 365). The census where revenue exactly covers all costs with nothing remaining. This is the survival floor — not the goal, not the destination. It is the minimum condition for the clinical team to be fully funded today.
10	Target Margin ADC	Annual Fixed Cost ÷ (365 × (Blended Revenue × (1 – Target Margin%) – Variable Cost)). The census where the branch hits its operating margin goal. This is the actual sales target — the number that closes the gap between surviving and sustainable.

Break-Even ADC — The Floor, Not the Goal

Break-even is the census level at which the branch covers every cost — salaries, medications, equipment, overhead — with nothing remaining. At break-even, every nurse is paid, every medication is funded, every supply is stocked. The patients currently enrolled are receiving care.

What break-even cannot pay for: merit raises, clinical education, quality improvement programs, equipment upgrades, recruiting bonuses to attract experienced clinicians, or any buffer against a sudden census drop. Break-even is survival. It is not sustainability. It is the floor — the minimum condition for the team to exist today. It says nothing about whether the team will still be whole tomorrow.

TYPICAL BREAK-EVEN RANGE

ADC 29 – 40 (Base Preset Assumptions)

Using national base RHC rates, standard variable costs, and a lean but complete staffing model, most new hospice branches hit break-even somewhere between ADC 29 and ADC 40 — depending on overhead structure, local wage rates, and patient acuity. Below break-even, the branch is losing money on every patient-day. The clinical team is present and serving — but the business is drawing down capital that will eventually run out. Break-even is not a finish line.

Target Margin ADC — The Real Sales Goal

Every hospice branch operates with a target operating margin — typically 12–18% of annual revenue. This margin is not profit extracted from patient care. It is the financial cushion that allows the organization to do the things that break-even cannot: replace aging equipment before it fails a patient, retain experienced clinical staff who could earn more elsewhere, build out into underserved markets where families have no access to quality hospice, and survive a census disruption — a pandemic, a regulatory change, a key referral source lost — without immediately cutting the clinical team.

The Target Margin ADC is the census required to hit that margin goal. It is always higher than break-even — often by 15 to 25 patients. The gap between break-even and target margin ADC is the sales gap. It is the specific number of additional patients whose admissions move the branch from surviving to thriving — from a team that is funded today to a team that is protected for tomorrow.

"The gap between break-even and target margin ADC is the sales gap. That is the number your work closes. That is what your admission activity means."

Monthly Admissions — Maintaining the Flow

ADC is a stock — the count of patients currently on service. Admissions are the flow that maintains it. Every month, patients leave the census through death, discharge, or revocation. The number of new admissions required to replace that attrition and maintain the target ADC is determined by a single formula:

FORMULA

Example: Target ADC 60, Average LOS 90 days = $(60 \times 365) \div 90 \div 12 = 20.3$ admissions/month ≈ 4.7 /week

This is not a quota a manager invented. It is the mathematical rate at which new patients must enter service to replace the patients who are dying and being discharged at the other end. Every missed admission is a gap in that flow. And gaps in that flow do not stay abstract. They show up as a nurse who cannot be hired, an aide whose caseload grows beyond what is safe, a chaplain whose hours get cut — and eventually, a patient who does not receive the care they were promised.

Required Admissions by Target ADC — 90-Day Average LOS

TARGET ADC	PATIENT-DAYS / YEAR	MONTHLY ADMISSIONS	WEEKLY ADMISSIONS	MARKETERS NEEDED @8/mo	EST. ANNUAL REVENUE
30	10,950	~12 / month	~3 / week	~2	~\$2.1M
50	18,250	~20 / month	~5 / week	~3	~\$3.6M
60	21,900	~24 / month	~5–6 / week	~3	~\$4.3M
80	29,200	~32 / month	~7–8 / week	~4	~\$5.7M
100	36,500	~41 / month	~9–10 / week	~5	~\$7.1M

Assumes 90-day average LOS. Revenue estimates use blended rate ~\$195/patient/day. Use the Spartan simulator with your actual rates for precise figures.

THE ADC 60 REFERENCE POINT

A branch targeting ADC 60 with a 90-day average LOS needs approximately 24 admissions per month — roughly 5–6 per week. That is the mathematical rate at which new patients must enter service to replace the patients who are dying and being discharged at the other end. Every missed admission is a gap in that flow. Every gap in that flow is a nurse who cannot be hired, an aide whose caseload grows beyond what is safe, a chaplain whose hours get cut — and eventually, a patient who does not receive the care they were promised.

PART FOUR

Reading the Field Differently

Now that you understand what the number means, how the math works, and who depends on both.

Most hospice sales reps walk into a referral source's office thinking about their own number — how many admissions they need to hit their quota, what their manager will say if they come up short, whether this account is worth the time. The rep who has read this far walks in thinking about a different set of numbers — and a different set of people. That shift changes everything about the conversation.

What Changes When You Know This

You stop selling a benefit and start funding a team.

The difference between a branch at ADC 45 and ADC 65 is not an administrative metric. It is the difference between a branch that can afford its after-hours RN and one that cannot. Between a branch that can retain its most experienced case manager and one that loses her to a competitor with a more stable census. Between a branch that can send its chaplain to additional training and one that cuts his hours. Every admission you generate is a contribution to that team's ability to function at full capacity — not just today, but sustainably.

Your referral source's hesitation costs someone a care team.

When a physician delays a hospice referral, or when a facility liaison is noncommittal about referring to your branch, the patient who would have benefited is the one who pays. Not in dollars — in care. In the nights they spend in pain that could have been managed. In the conversations they don't get to have because no one with the training and the time shows up to facilitate them. The family that needed a social worker to walk through the room. The patient who needed a chaplain. The exhausted adult child who needed someone to tell her she didn't have to do this alone.

Your own follow-through is a clinical act.

The admission that didn't happen because you didn't follow up is a patient who didn't get enrolled. That is a clinical outcome, not a sales outcome. Reps who understand this don't need to be motivated to follow up. They understand the weight of not doing it. The follow-up call is not about hitting a number. It is about whether a specific family has access to the most influential people in the dying process — or doesn't.

Your activity targets have a why.

Five admissions per week. Twenty per month. These are not arbitrary activity standards invented by leadership. They are the admission rate required to hold a census that sustains the clinical team at full strength. Every referral conversation you have either moves that rate forward — or leaves the gap in place. The gap is not a number on a report. It is a nurse position that doesn't get filled, a patient whose aide is

stretched too thin, a chaplain who isn't there.

Break-even is not the destination. Sustainability is.

A branch at break-even is surviving. It can fund the team today. It cannot invest in them tomorrow. Cannot attract and retain the most experienced clinicians. Cannot expand into underserved communities where families have no access to quality hospice care. Cannot absorb the disruptions — the pandemic, the staffing crisis, the reimbursement change, the key referral source who retires — that will inevitably come. The gap between your current ADC and target margin ADC is the gap between a branch that exists and one that grows.

"You are not closing a sale. You are connecting a patient to a team of people who have trained their entire careers for this specific moment in that patient's life."

What to Say When You Know This

You do not need to lead with profitability math in a referral conversation. You lead with the clinical team. But knowing the math gives you the authority to say what follows with the conviction that comes from actually understanding it — not performing it.

On clinical capacity:

"Our branch has the clinical depth to handle complex patients because our census allows us to staff for it. An RN Case Manager for every 12 patients on service. Dedicated after-hours RN coverage every night of the year. That is not every branch."

On family support:

"The families who come to us trust us with the most important time of their lives. We take that seriously enough to make sure our census supports the staffing to do it right — not the minimum staffing required to keep the doors open."

On the referral itself:

"Every patient we enroll helps us maintain the team that serves every other patient. Every patient we cannot reach is one fewer family with access to what we can provide."

On timely referral:

"Earlier enrollment means more days of expert symptom management, more time for the social worker to get ahead of the family's needs, more opportunity for the chaplain to be present. The patient who enrolls at 60 days has a fundamentally different experience than the patient who enrolls at 6."

None of these statements are marketing language. They are true. They are grounded in the math and the mission this document has described. When you say them, you say them as someone who understands what they mean — and referral sources who have seen enough reps can tell the difference.

THE SPARTAN SIMULATOR

A Live Model of Your Branch's Financial Reality

Built on exact formulas. Updated in real time. Specific to your rates, costs, and market.

Every number in this document can be calculated for your specific branch using the Spartan Branch Profitability Simulator — an interactive financial model built directly into the Spartan Coaching platform. It is not a spreadsheet approximation. Every calculation uses precision arithmetic applied to the exact formulas described in Part Three, with your actual revenue rates, clinical costs, staffing structure, and operating assumptions.

What the Simulator Computes

INPUT CATEGORY	WHAT YOU ENTER	WHAT IT GENERATES
Census & LOS	Target ADC, Average Length of Stay	Blended revenue per day, monthly and weekly admissions required to maintain census
Revenue Rates	RHC Day 1–60 rate, RHC Day 61+ rate	Annual revenue at target ADC, exact blended per-diem for your patient mix
Variable Costs	Pharmacy, DME, supplies, travel, other — per patient per day	Contribution margin per patient per day, annual variable cost
Staffing Structure	All staff roles with salary inputs	Annual payroll, scaling analysis, required staff count at target ADC
Operating Overhead	Monthly non-payroll fixed costs	Annual overhead, full fixed cost total
Target Margin	Desired operating margin %	Target Margin ADC — the census required to hit your financial goal
Starting Capital	Cash available at launch	18-month cash runway projection showing break-even crossing month
Sales Assumptions	Admissions per marketer per month	Number of marketers required to generate the admissions needed at target ADC

How to Read the Output

Profit & Revenue Chart

A continuous curve showing annual profit across the full ADC range (10–200). Break-even ADC and Target Margin ADC are marked on the curve. You can see immediately how far above break-even your current or target census sits.

Operating Margin Curve

The margin percentage at every census level. Watch for the 10% 'fragile' threshold and the 12–18% 'healthy' band. A branch hugging 10% has no buffer. A branch at 15% can absorb a 10-patient census drop without cutting staff.

Required Staffing Table

Every role required at your target ADC, with annual cost and headcount. Shows exactly how many RN Case Managers and Hospice Aides are required and what the full payroll looks like at that census level.

Cash Runway Projection

An 18-month month-by-month model showing cumulative cash position as census ramps toward break-even. Identifies the break-even crossing month and how much starting capital is required to reach it.

Admissions Needed

Monthly and weekly admission requirements at your target ADC and average LOS. Also calculates the number of marketers required at a given admissions-per-marketer assumption — so you can understand what your sales team needs to look like.

HOW TO USE THE SIMULATOR IN THE FIELD

Run your branch's actual numbers before your next manager conversation. Know your break-even ADC. Know your target margin ADC. Know your monthly admissions requirement. Know your cash runway crossing month.

Then use those numbers not to impress your manager — but to understand exactly what your daily work is building toward, and what the team you're funding looks like at each census milestone.

Access the simulator at: spartanhospicecoaching.com → Tools → Branch Profitability

CLOSING

The Census Is Not the Point. The Care Is.

The only thing a hospice sales rep needs to remember.

The census number you were given at the beginning of your career was not arbitrary. It was never arbitrary. It was the financial expression of a specific human requirement: the number of patients your branch needed to serve in order to sustain the team of people who would provide that service.

That team — the nurses, the aides, the social workers, the chaplains, the after-hours clinician, the intake coordinator who answered on the first ring — exists for one reason. Not to generate revenue. Not to hit a margin goal. To be present with people in the most vulnerable, most sacred, most irreversible chapter of their lives. To do the work that requires both professional training and human courage: to walk into a room where someone is dying and to make that room, for a time, less frightening.

The census is what makes that possible. Your work is what builds the census. That connection — between the call you made this morning and the chaplain who sat with a patient tonight, between the referral you followed up on and the aide who arrived with dignity and competence — is not metaphorical. It is direct. It is causal. It is real.

The most influential people in the dying process are there because someone built the census that funds them. When you understand that, the number is never arbitrary again.

"The census is not the point. The care is the point. The census is the condition that makes the care possible."

A FINAL NOTE

This document is part of the Spartan Coaching Branch Profitability Education Series. The Spartan Branch Profitability Simulator — accessible at spartanhospicecoaching.com — computes every number in this document for your specific branch, using your actual rates, costs, and operating assumptions. It is free to use and takes approximately three minutes to complete. Run your numbers. Know your floor. Know your goal. Know why it matters.

KEY TERMS

Glossary

Plain-language definitions for every metric and formula in this document.

Average Daily Census (ADC)

The number of patients actively enrolled in and receiving hospice care on any given day. The single most important operational metric in hospice — the starting point for every financial calculation and the measure of a branch's capacity to serve its community.

Routine Home Care (RHC)

The standard Medicare hospice level of care, paid as a flat daily per-diem rate. Two rates apply: Day 1–60 (\$224.62/day at 2025 national base) and Day 61+ (\$177.27/day). Actual rates vary by geographic wage index. These per-diems fund every service the branch provides — clinical visits, medications, equipment, and coordination.

Contribution Margin Per Day

Blended RHC per-diem minus direct variable costs per patient per day. This is the amount each enrolled patient contributes toward the fixed costs (payroll and overhead) that sustain the clinical team. Formula: Blended Revenue Per Day – Total Variable Cost Per Day.

Blended Revenue Per Day

The weighted average per-diem across a patient's enrollment, accounting for the mix of Day 1–60 and Day 61+ rates based on average length of stay. For LOS > 60 days: $((\text{Day1Rate} \times 60) + (\text{Day61Rate} \times (\text{LOS} - 60))) \div \text{LOS}$.

Break-Even ADC

The census level at which total revenue exactly equals total costs, leaving zero profit. Formula: $\text{Annual Fixed Cost} \div (\text{Contribution Margin Per Day} \times 365)$. This is the survival floor — not the goal. At break-even, the clinical team is fully funded today. It cannot be retained, expanded, or protected tomorrow.

Target Margin ADC

The census level at which the branch hits its operating margin goal. Formula: $\text{Annual Fixed Cost} \div (365 \times (\text{Blended Revenue} \times (1 - \text{Target Margin}\%) - \text{Variable Cost}))$. This is the actual sales target — the number that closes the gap between surviving and sustainable.

Operating Margin %

$\text{Annual Profit} \div \text{Annual Revenue} \times 100$. The industry benchmark for a financially healthy hospice branch is 12–18%. Below 10% is considered financially fragile. This margin funds equipment replacement, staff retention and raises, quality improvement programs, and the financial buffer needed to absorb census disruptions.

Cash Runway

The number of months a branch can sustain operations from starting capital before reaching break-even. Calculated using a linear ADC ramp model over 18 months. A planning tool — actual runway depends on census ramp speed, hiring timing, and market conditions.

Monthly Admissions Needed

The number of new patient admissions required per month to maintain a target ADC. Formula: $(\text{Target ADC} \times 365) \div \text{Average LOS (days)} \div 12$. This is the flow required to replace patients who are dying or being discharged at the other end of the census — not an activity standard invented by management.

Medicare Conditions of Participation

The federal regulatory requirements a hospice must meet to maintain Medicare certification. Includes mandatory staffing ratios (1 RN Case Manager per 12 patients; 1 Hospice Aide per 8 patients), required service disciplines (social work, chaplaincy, medical director oversight), and quality and documentation standards. These requirements exist whether the branch has 15 patients or 150.
